SRR - Site Risk Register

SRR - v0.1

Site Name / Code	Protocol Number	Principal Investigator	Register Owner / Reviewer
Date Opened	Current Review Date	Study Phase	Site Type / Implementation Level

A Purpose and Use of This Register

The Site Risk Register is the site-level record used to identify, prioritise, monitor, and periodically reassess risks arising from local execution of the clinical investigation. It does not duplicate the sponsor's trial-level risk assessment or the manufacturer's device risk file. It captures risks shaped by site staffing, workflow, infrastructure, surgical execution, device traceability, documentation, follow-up reliability, imaging practice, and escalation discipline.

The register should focus on risks that may affect participant safety, procedural integrity, device accountability, endpoint interpretability, or reliability of site-generated data. Each entry should be linked to a CtQ domain, existing controls, review trigger, risk owner, and PI escalation requirement, and should be updated during scheduled reviews or when meaningful new information arises.

B Risk Categories

Risk Category	Typical Site-Level Risk Examples	Main Reason for SRR Entry
Safety / clinical event	Delayed SAE awareness, incomplete ADE assessment, unclear device/procedure relationship, missed escalation.	May affect participant protection or timely clinical decision-making.
Procedure / surgeon performance	Learning curve drift, unplanned component substitution, inconsistent operative documentation, complexity mismatch.	May affect procedural consistency, causality, or endpoint interpretation.
Device accountability	Missing UDI or lot number, unreconciled component, wrong implant configuration record, return/disposal gap.	May break participant-device traceability or inspection readiness.
Data / endpoint reliability	Missing critical imaging, inconsistent PROM/ROM collection, unresolved critical query, source-to-CRF mismatch.	May affect reliability, completeness, or interpretability of critical data.
Operational / staffing	Single point of failure, training gap, delegation mismatch, turnover, overloaded coordinator workflow.	May create process drift or unclear ownership of critical tasks.
Consent / eligibility	Wrong ICF version, late reconsent, borderline eligibility, missing eligibility source evidence.	May affect participant rights, enrollment validity, or regulatory compliance.
Imaging / assessment workflow	Non-standard image acquisition, missed image window, incomplete functional assessment, assessor inconsistency.	May affect endpoint quality or comparability across subjects.
Vendor / external data	Delayed external records, missing readmission data, external imaging not transferred, ePRO access issue.	May hide safety events or create incomplete endpoint evidence.

Common Review Triggers

[ ] Repeated protocol deviations	[ ] Missed critical imaging or endpoint window	[ ] Delayed SAE / ADE reporting
[ ] Device accountability discrepancy	[ ] Learning curve or surgeon performance signal	[ ] Monitoring finding or audit observation
[ ] CAPA recurrence or ineffective CAPA	[ ] Staffing, delegation, or training change	[ ] Protocol amendment or workflow change

C CtQ Domains Available for SRR Linkage

No.	CtQ Domain	What the SRR Should Protect
1	Participant selection	Correct eligibility, appropriate clinical indication, and adequate supporting source evidence.
2	Informed consent	Correct version, timing, reconsent, comprehension, and documentation completeness.
3	PI oversight	Timely PI involvement where clinical judgment, safety, or critical data interpretation is required.
4	Delegation and training	Qualified personnel, role boundaries, documented training, and controlled task assignment.
5	Surgical technique	Protocol-consistent procedural execution and control of surgeon-dependent variability.
6	Learning curve	Detection of early procedural drift, repeated technical difficulty, or competence-related signals.
7	Device accountability	UDI, lot, implant configuration, receipt, use, return, disposal, and participant-device traceability.
8	Safety surveillance	AE/ADE/SAE/device deficiency detection, documentation, relatedness review, and timely reporting.
9	Protocol deviations	Detection, explanation, reporting, recurrence control, and CAPA linkage for significant deviations.
10	Imaging quality	Protocol-compliant image acquisition, transfer, completeness, timing, and interpretability.
11	Endpoint data reliability	Completeness and consistency of PROM, ROM, imaging, clinical outcome, and source-to-CRF evidence.
12	Follow-up retention	Visit-window control, missed assessments, lost-to-follow-up prevention, and recovery actions.

D Risk Rating Guide

Rating Field	1	2	3
Probability	Low: Unlikely or isolated vulnerability.	Medium: Plausible during normal site conduct.	High: Likely, recurrent, or already observed.
Safety	No expected safety effect or administrative concern only.	May affect timely clinical review, participant protection, or safety documentation.	May cause or obscure SAE/ADE, device deficiency, clinically relevant harm, or urgent PI decision.
Procedure	No expected effect on procedure conduct.	May affect procedural consistency, documentation, complexity handling, or protocol adherence.	May affect surgical appropriateness, intraoperative decision-making, technical execution, or causality interpretation.
Device Accountability	No device traceability concern.	Minor discrepancy or delayed reconciliation with recoverable evidence.	Missing or conflicting UDI, lot, component, implant configuration, return/disposal, or participant-device link.
Data Reliability	No expected effect on critical data.	May affect completeness, timeliness, or source-to-CRF consistency.	May compromise endpoint interpretation, eligibility validity, safety analysis, or inspection-critical source evidence.
Detectability	Easy: Detected before harm, data loss, or missed reporting.	Moderate: Likely detected during planned review, monitoring, or reconciliation.	Difficult: May remain hidden until endpoint review, inspection, or event recurrence.

Low	Moderate	High	Critical
All impact dimensions are 1, probability is low, and the risk is easily detectable. No PI escalation normally required.	At least one impact dimension is 2, or probability is medium, but no impact dimension is 3 and controls appear adequate.	Any impact dimension is 3, or the issue is recurrent, poorly controlled, difficult to detect, or requires active PI review.	Immediate or unresolved safety risk, unreconciled device traceability break, unrecoverable endpoint-critical data loss, or reportable/escalation event.

E Worked Example - How One Risk Entry Should Be Completed

Example Section 1 - Identification and Rating

Risk ID	Date Opened	Risk Category	Risk Statement	CtQ Domain	Prob.	Safety	Proc.	Device	Data	Detect.	Priority
SRR-003	2026-06-28	Device accountability	Implant lot number or UDI may not be captured and reconciled in both source record and device accountability log before the OR workflow closes.	Device traceability / participant-device link	2	2	2	3	3	2	High

Example Section 2 - Controls, Ownership and Follow-Up

Risk ID	Existing Controls	Root Cause / Vulnerability	Owner	PI Escalation Required?	Trigger / KPI for Review	Planned Mitigation	Status
SRR-003	OR implant stickers; device accountability log; coordinator same-day reconciliation.	UDI evidence moves between OR team, operative note, and research file; no second-person verification currently documented.	Site Quality Representative / Study Coordinator	Yes, if missing or conflicting UDI remains unresolved.	Any missing lot/UDI; any UDI mismatch; more than one reconciliation discrepancy in a review cycle.	Add OR closeout checkpoint; require second-person verification; reconcile before CRF entry.	Open - mitigation active

In this example, root cause is refined after review of source records, workflow evidence, monitoring findings, or CAPA investigation.

F Main Site Risk Register - Identification and Rating

Risk ID	Date Opened	Risk Category	Risk Statement	CtQ Domain	Prob.	Safety	Proc.	Device	Data	Detect.	Overall Priority

Use this page first to define and prioritise the risk. Continue the same Risk ID on the next page.

Additional Notes / Manual Observations

G Main Site Risk Register - Controls, Ownership, and Follow-Up

Risk ID	Existing Controls	Root Cause / Vulnerability	Risk Owner	PI Escalation Required?	Trigger / KPI for Review	Planned Mitigation / Action	Linked Records	Status	Last Review	Next Due	Init.

Linked records may include POT, MRL, CSST, CAPA, deviation log, device accountability, monitoring finding, imaging log, or source review note.

Additional Notes / Manual Observations

H Risk Review Log

Review Date	Risk ID	New Evidence / Signal Reviewed	Rating Changed?	Action / Control Changed?	PI Decision / Escalation	Next Review Due

I Emerging / Urgent Risk Entry

Date / Time Identified		Detected By / Role	
Immediate Category	[ ] Safety [ ] Procedure [ ] Device accountability [ ] Data / endpoint [ ] Consent / eligibility [ ] Imaging [ ] Follow-up [ ] Other		
Initial Risk Description			
Immediate Containment / Safety Action			
PI Notified?	[ ] Yes [ ] No [ ] Pending	Formal SRR Entry Required?	[ ] Yes [ ] No [ ] After review
Disposition / Next Step			

J Downgrade, Escalation, and Closure Criteria

Decision Type	Minimum Evidence Expected	PI Review Needed?	Record Link
Downgrade priority	Controls implemented; trigger has not recurred for the defined review period; no unresolved safety, device accountability, endpoint, or data reliability concern remains.	Yes for High/Critical risks; optional for Moderate risks.	
Close risk	Risk no longer applies, study phase has ended, workflow changed, or mitigation has been effective with documented evidence and no open linked action.	Yes, if risk ever affected safety, device traceability, endpoint data, or CAPA.	
Escalate to CAPA	Risk has materialised, recurred, breached threshold, or shows ineffective control. Root cause and corrective/preventive action are required.	Yes.	
Escalate to POT / MRL	Risk requires PI decision, milestone review, unscheduled review, or change in oversight intensity.	Yes.	
Escalate to sponsor / CRO	Risk affects reportable safety event, significant deviation, device deficiency, endpoint integrity, monitoring commitment, or sponsor-defined escalation threshold.	Yes, according to CIP and site procedure.	

K Risk Register Review Certification

PI Certification Statement	I confirm that the site risks documented in this register have been reviewed at a level proportionate to their potential impact on participant safety, procedural integrity, device accountability, and reliability of site-generated data. I confirm that required escalations, CAPAs, mitigation actions, or follow-up reviews have been identified or documented where applicable.		
Principal Investigator Signature		Date	
Register Owner / Reviewer Signature		Date	
Open High / Critical Risks Remaining			
Next Scheduled SRR Review		Linked MRL Review ID	